

1. Administrative & Study Identification

Full Study Title: A Phase I Dose Escalation and Expansion Study to Evaluate the Safety, Tolerability, Pharmacokinetics and Preliminary Clinical Activity of R07673396 as a Single Agent and in Combination With Other Anticancer Therapies in Patients With Advanced Solid Tumors Harboring RAS Mutation(s) **Short Title/ Acronym:** R07673396 in Advanced Solid Tumors **Protocol Number:** Y045758 **NCT Number:** NCT06884618 **Sponsor Name:** Hoffmann-La Roche Ltd. **Investigational Product:** R07673396 (pan-KRAS inhibitor, oral tablets) **Phase of Development:** Phase I **Medical Monitor:** Roche Clinical Operations Lead / Safety Review Committee (SRC)

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety and tolerability of R07673396 and to determine the recommended dose(s) for expansion (RDEs) in participants with advanced solid tumors harboring RAS mutation(s). **Secondary Objectives:** To evaluate the preliminary anti-tumor clinical activity of the RDE(s) as a single agent and in combination with other therapies, and to assess pharmacokinetics (PK) and pharmacodynamics.

2.2 Background & Rationale To address the critical medical need in treating advanced, incurable solid tumors that harbor RAS mutations, which are notoriously difficult to treat. This study tests R07673396, a novel pan-KRAS targeted inhibitor, to determine if a highly specific targeted approach can reduce reliance on broader, less targeted therapies while delivering better clinical outcomes and manageable side effects for patients.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm (for the primary monotherapy dose-escalation phase) **Blinding:** Open-label **Randomization:** N/A (Dose escalation)

3.2 Planned Enrollment & Duration Target \$N\$: 345 **Number of Sites:** Global multicenter (Including sites in the US, Australia, Canada, New Zealand, Singapore, and Taiwan) **Estimated Study Start:** 30-Apr-2025 **Estimated Primary Completion:** 31-Aug-2028

4. Subject Selection (Eligibility Criteria)

- 4.1 Inclusion Criteria** 1. Histologically documented, locally advanced, recurrent, or metastatic incurable solid tumors. 2. Confirmed presence of the RAS mutation(s). 3. Participants with measurable disease according to RECIST v1.1 as assessed by the investigator. 4. Eastern Cooperative Oncology Group (ECOG) performance status of 0 or 1. 5. Life expectancy ≥ 12 weeks with adequate hematologic and end-organ function.
- 4.2 Exclusion Criteria** 1. Known and untreated, or active central nervous system (CNS) metastases. 2. Refractory nausea and vomiting, malabsorption, external biliary shunt, or significant small bowel/gastric resection precluding adequate study treatment absorption. 3. Treatment with chemotherapy, immunotherapy, biologic therapy, or an investigational agent as anti-cancer therapy within 4 weeks or five half-lives prior to initiation of study treatment. 4. Major surgical procedure within 28 days prior to initiation of study treatment.
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5. Intervention & Treatment Plan

- 5.1 Investigational Regimen Dosage/Frequency:** Multiple ascending doses of R07673396 as per a pre-defined dosing regimen (Stage 1 determines RDE; Stage 2 evaluates RDE). **Route of Administration:** Oral (Tablets). **Duration of Treatment:** Administered until unacceptable toxicity, disease progression, and/or loss of clinical benefit as determined by the investigator.
- 5.2 Concomitant & Prohibited Therapy Permitted:** Standard supportive care for symptomatic management. **Prohibited:** Concurrent treatment with other investigational agents or recent anti-cancer therapies within 4 weeks prior to initiation.
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6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Mutation Confirmation, Baseline Scans
Baseline	Day 1	First Dose, PK/PD Baseline Sampling
Interim	Cycles 1+	Safety Labs, PK/PD Sampling, Tumor Assessments per RECIST v1.1
End of Study		Disease Progression / Toxicity Final Vital Signs, End of Treatment Assessment, Survival Follow-up

7. Outcome Measures (Endpoints)

- 7.1 Primary Endpoint Definition:** Number of participants experiencing Adverse Events (AEs) and Dose-Limiting Toxicities (DLTs) to determine safety, tolerability, and the Recommended

Dose for Expansion (RDE). **Measurement Method:** Clinical assessment and laboratory tests graded by CTCAE.

7.2 Secondary & Exploratory Endpoints Preliminary anti-tumor activity (including Objective Response Rate [ORR] and Progression-Free Survival [PFS] per RECIST v1.1). Pharmacokinetic (PK) parameters (e.g., Cmax, AUC).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring via regular clinical evaluation and routine safety labs throughout dose-escalation and expansion. **Serious Adverse Events (SAEs):** Standard oncology reporting timeline, typically within 24 hours of investigator awareness. **Data Monitoring Committee (DMC):** An internal Safety Review Committee (SRC) will evaluate DLTs and guide dose-escalation decisions.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (all participants receiving at least 1 dose of study drug); Efficacy-Evaluable Population. **Sample Size Justification:** Sample size driven by standard Phase I dose-escalation operating characteristics rather than strict power calculations; expansion cohort size determined by preliminary efficacy signals. **Handling of Missing Data:** Standard oncological methods; survival and progression data will be censored at the last evaluable tumor assessment.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular onsite and remote monitoring visits for Source Data Verification (SDV) and safety reviews. **Audit Trail:** eCRF locking, tracking of protocol deviations, and data clarification forms via standard clinical data management systems.

11. Study Identifiers & Governance

Primary ID: Y045758 **Registry Number:** NCT06884618 **Sponsor/Lead Organization:** Hoffmann-La Roche Ltd. **Study Title:** R07673396 in Participants With Advanced Solid Tumors Harboring RAS Mutation(s) **Official Regulatory Title:** A Phase I Dose Escalation and Expansion Study to Evaluate the Safety, Tolerability, Pharmacokinetics and Preliminary Clinical Activity of R07673396 as a Single Agent and in Combination With Other Anticancer

12. Clinical Framework (Oncology/RAS Mutation Specific)

Primary Condition: Advanced Solid Tumors **Diagnosis Threshold:** Confirmed presence of RAS mutation(s) **Medical Methodology:** Targeted therapy (pan-KRAS inhibitor) **Optimization Target:** Maximum Tolerated Dose (MTD) / Recommended Dose for Expansion (RDE)

13. Study Procedures & Methodology

Management Pathway: Oncology clinical pathway for advanced solid tumors **Education Protocol (HCP):** Site Initiation Visits (SIV) and protocol/dose-escalation specific training **Education Protocol (Patient):** Informed consent process and oral dosing compliance training **Quality Audit Mechanism:** Clinical site monitoring, Source Data Verification, and Safety Review Committee (SRC) oversight

14. Observation & Follow-Up Schedule

Total Duration: Until disease progression, unacceptable toxicity, or loss of clinical benefit **Onsite Visit Cadence:** Cycle 1 (frequent for PK/safety), then generally Day 1 of subsequent 21/28-day cycles **Remote Monitoring Frequency:** Ongoing as per clinical site monitoring plan **Checklist Review Interval:** Tumor assessments every 8 weeks (per RECIST v1.1)

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	Erin Schenk, MD, PhD	_____	03-Apr-2026	Clinical Operations Lead	Levi Garraway, MD, PhD	_____	03-Apr-2026
Lead Statistician	Hoffmann-La Roche			Biostatistics Lead	_____	03-Apr-2026	